भारत सरकार
IPC

SUSPECTED ADR REPORTING

PvPI Helpline (Toll Free) :1800-180-3024 (9:00 AM to 5:30 PM, Monday-Friday) -Virtual works Lab Assignment

CASE NARRATIVE

A 45-year-old female receiving anti-tubercular therapy developed symptoms and biochemical evidence suggestive of hepatotoxicity approximately three weeks after treatment initiation. The suspected treatment was temporarily discontinued following clinical assessment, after which the patient's symptoms and liver function parameters improved. Based on the temporal relationship, known pharmacological association, positive dechallenge and absence of a stronger alternative explanation, the reaction was assessed as probable/likely. The case was documented as a serious suspected ADR and submitted for pharmacovigilance evaluation.

Paitent Details

Patient initials: M.S
Age (years) : 45
Sex : F
Weight (Kg) : 58

Relevant medical history: Pulmonary tuberculosis
Known allergies: No known drug allergies
Relevant past medical history: No previous history of liver disease
Alcohol history: No significant consumption

Suspected Adverse reaction

Reaction Start Date: 1.10.2026
Reaction Stop Date: 22.10.2026

Details of the reaction

The patient was clinically stable during the initial period of treatment. Approximately 3 weeks after initiation of anti-tubercular therapy, she developed: Loss of appetite, Nausea, Generalized weakness, Abdominal discomfort, Dark-colored urine, yellowish discoloration of the eyes.

Case Identification

Case Report No.: ADR/INT/2026/001
Date of Report: 27.10.2026
Reporting Source: Hospital Pharmacovigilance/ADR Monitoring Unit
Type of Report: Initial report
Source of Report: Healthcare professional
Department: Medicine/Pharmacy
Case Classification: Suspected adverse drug reaction

Suspected Medication

DRUG	DOSE	ROUTE	FREQUENCY	INDICATION
Isoniazid	300mg	Oral	Once Daily	Tuberculosis
Rifampicin	600mg	Oral	Once Daily	Tuberculosis
Pyrazinamide	1600mg	Oral	Once Daily	Tuberculosis
Ethambutol	800mg	Oral	Once Daily	Tuberculosis

Concomitant medications

No concomitant hepatotoxic medication was reported at the time of initiation.

ADR Assessment

Suspected ADR: Drug-induced hepatotoxicity associated with anti-tubercular therapy.
Seriousness: YES (hospitalization/required significant medical intervention.)
Expectedness: Hepatotoxicity is a recognized adverse effect associated with some anti-tubercular medicines, particularly isoniazid, rifampicin and pyrazinamide.
Dechallenge: Positive
Rechallenge: Not Performed

Causality Assessment

WHO-UMC-style assessment
Causality category: Probable/Likely
Justification:
The adverse reaction occurred after initiation of the suspected medicines, has a medically plausible temporal relationship, is consistent with the known safety profile of the therapy, and showed improvement following withdrawal. Rechallenge was not performed.

OUTCOME

Following withdrawal of the suspected medicines, the patient's symptoms gradually improved and liver enzyme levels decreased.
Outcome: Recovering/improving at the time of follow-up.
No fatal outcome was observed.

REPORTERS DETAILS

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